

00:06

So we only established that through the legislative limitations, we can't legally diagnose with this plan for medical advice. But taking that into consideration, first of all, should we operate on the assumption that we have gotten permission from the government? And the second option is that we could divide this based on whether, just based on location, since we might have access for some countries, but we might not have access for others, maybe to My point of view on that is that, let's say this is a real thing. Yes, we need permission from the government because this is citizen's data.

00:58

There needs to be like, you know how like, places look, there needs to be like some sort of way to like, kind of, like we need to make all the bigger countries more powerful, more powerful countries. Like, and it's like a true saying, okay, we're not going to be like, we're not launching any cyber security attacks, we're not launching anything. And then we need to like, you know, make sure that, you know, we have cyber security and everything. Cyber security, yeah.

01:25

whether or not, I'm blanking here, we need to also ensure that we are approaching, because we're only offering medical advice, and we are operating under, because a lot of countries tend to like, I don't know, like we're giving objective medical advice and that, and I don't know, maybe even like partner with like medical organizations across like international medical organizations, but that's another thing for later. But I think your key assumption is, could work. Because right now we're describing like-- So you want to divide it, or you want to assume we get permission? Like divide it for-- you just divide it based on the permission system and just add the migration. Yeah, we need to flat. We need to divide it based on permission system, because-- you know? Sorry.

02:23

I think we should divide it, but I also think it has overhead. But I also don't know. I'm going to get pissed on that. Some governments are not going to be comfortable with a tech company based in the US. We're not a company. My idea for this product is that we are essentially providing WHO, which is a free organization, the World Health Organization, that has a ton of influence across the healthcare sector. across all countries in the world. When did we first look at who supports tech initiatives for our scalpers? Maybe there's some other party we're not looking at. There are a lot of venture capitalists. Isn't it cool first? But a lot of people just do it for profit. Not for non -profit. For non -profit, WHO is connected to all non -profits, essentially. Yeah, but we should still go into which one more than that. Okay, we will do that.

03:23

We will do that. Okay, so, but we should not. For the... But the permission -based system. Do they have access? Can we do this? Can we provide medical advice? If we can't do that, we can do this shit. Let's look at the WHO documentation of what they define as providable service, healthcare service, without... Okay, can you first look at that before we implement that? So now I guess I can start working on the conversation. Are we going to use the chatbot or are we going to use this... TTS? I feel like, I mean we're gonna use TTS anyway. So you want sex conversation? I wanna have two modes of the user that they can interact with us. One, the phone call, they just dial 150 or something and the person can get on a call with an agent that talks back and does everything that we just talked about.

04:18

and also an SMS system where they can upload photos. They can also upload photos while they're talking to the person. The agent can be like, Hey, can you upload a photo of this? And the person can upload the photo to a link. - Okay, so you want two APIs? One that does the speech? - Does the, yes. - Does the, both the exact, okay. So I can start working on that, I guess. - Yeah. - Before we-- - My reasoning for this is, phone calls and SMSs already have a ton of encryption on them. Their encryption technology is very high. So we don't have to think about separate encryption for that service. That was my thought process on that. Yeah.

I'm somehow wondering, assuming they have access to a phone and everything. They have a phone, man. They have a phone. I'm just saying. I mean, maybe I'm just being ignorant.

05:19

Let's take a wild guess and say, yeah, we need to look for statistics. They say if they're sitting at a gym, they'll have access to a phone. Also, just to clarify, the LN1 would be able to see the photo, right? No. So, do you want to work on the technical side or on the business side? Or like the presentation side? Well, I think... much faster than the presentation. Because then, since I'll-- do you guys know about this, the Facecom? The organizers? Where were the organizers?

06:16

I think Japanese food, Korean food. Oh, wow. I can take it whenever you want. You'll just have to show off your friend. I asked. Believe me, I've asked. It's like, oh, you're just as safe as a smooch by 10 minutes. Who's a guy like sick on a Sunday? I'm going to try to get them all on one certain other. I know what I have to do. Permission granted. Permission granted. Okay, so I'll try sharing a vlog.

07:20

Just, where's my cue? Is it a female? What? It sounds like female. I am sending them the email. This is to this guy. I'm really sorry. Normally, when it's normal, I'm usually a few on the weekends, but it's like this time around. Well, you're good. It's like the bad luck is weird. Okay, see. What I prioritize is human insight over human...

08:14

Because with AI in the flow, we don't really need people all the time and so on. We just need people's insights so that we have a more holistic view about it. So I want to get the perspective of the business side, accounting perspective, and numbers, and different perspectives, essentially. Every person has a different way of thinking about it. Yeah. I'm not like, I'm not even sure how much this would cost because we don't have like, Assuming we're using anthropic systems, I'm not even sure how much this would cost. No, we're not using anthropic at all. We're using data centers. Oh, we're using data centers? Yeah, but that would be on the WHO. It's not on us. But we'll have to calculate how much data center they'll be using. How do we calculate that? I think I need help with that because I don't know if that means I don't have credit.

09:08

If you search up per phone call cost per minute per office hours in a certain country, it will tell you 0 .001 cent or something per second. We'll use that metric to give numbers. Also, I'll share you the document that Michelle is working on. Is it on the-- oh.

09:42

I'm trying to share our names so that you have-- you know who your teammates are that you can mail to and to the Those are our views. Okay. Let me send this email very quickly.

10:24

Dear Lucas, Due to a What was I saying, man? So have you had dinner yet? No, I'm just... It's not that good. It's not nice. I didn't get to dinner either, but I got dinner right now. Let's have a bite.

11:27

Actually, I can talk more with the... I was staying here. How much longer do you want to get? How long can you stay here? I can stay here forever. Until next Monday. I live just a minute away. No, because it's much later. I would be home because I don't want to go home too late. I would take care of all that. He will write for you. Don't worry. I can drop you if you want. How far are you? Half a minute. - What are you doing? - Why are you asking her for miles?

12:26

- I wanna know how far she lives. In the fastest format. I don't know the places. - Just ask her landmark. - I don't know, like downtown. - Downtown or college mall. - Oh, that's, oh you live, oh I know that place. - You live next to me? - No, she lives at, what's that place? Where is that? It's like a big apartment. Is it at home? It's the house. Oh! It's like... right next to the... what was it? Yes. The yakgwa? That's Korean traditional. We got the pro version. I mean, did you scan the... Yes.

13:31

Yeah, I'm trying to make a shared space. I don't know if it's possible. So do you guys upgrade to like pro version? What? Like the cloud? Why is it built to a GitHub repo for this? No. Usually, I mean, you can create a GitHub repo either way. Because, like, Ben even, like, made changes to it. I'm trying to find a way that I can share, though. Michelle, you're good at clock. Can we share a cool working space? I don't know. Because you've never worked with anyone? What are you supposed to be saying? You're so funny. I fuck with you. Wow.

14:37

So you said that you upgrade your transcript, but why I can't see the further-- So, Michelle, you updated the new link, right? You just gave it a new link? It's the same one, it's the same chat. Oh, it's okay, same chat. Okay, I can't create a cool open space on fraud. What I can do is make a GitHub repo. Yeah, let's look at that. Okay, which one? Seriously, I can, but...

15:43

I'm sure we can use also a Docker file. Docker? I have a Docker file. Yeah. You have a very interesting set of roommates or neighbors. I'll try sharing the space with you. We can work on it. Michelle will send us the document and we'll work on it later. Okay?

16:44

Okay, sounds good. I'll add you to the group chat and we can just keep texting. Yeah, I'll add you to the credits. That doesn't matter. I mean, you can go without credits. Okay, yeah. You'll probably get it later today. Probably later. Probably tomorrow, that's likely. So... I'm probably a research on the data center costs. And WHO. If you could go to WHO documentation and give us what they consider as a service that people can provide versus not, and what we need, permissions. What permissions do you require, and stuff like that. OK. applicability of our idea and stuff like that.

17:44

Where can I find it? The WHO website. Or you can just chat to APJ and it will find the sources for you. Actually, I was going to say, you can check out this one. Yeah, perplexity is way better at research. It's 100 times better than every other platform that is available. But okay, I'll text you and just get back to me on that and we're gonna begin. I'm gonna share a cool workspace and a document with you, okay? Yeah, I'll just put you on mute for a while. okay. i mean, we'll be having the call. if you have something to talk about, just text me. i'll get on the call back. whatever. alright. because i need to lock in.

18:45

alright, I'll see you. i'm locking in. i need to lock in. i'll see you in a minute. Alright fellas, Michelle to answer your question of when you need to go back home, when do you need to go back home? Do you have a co -fuel? I just don't like it. I mean, if I was with you, I would go alone for a month. Alone for a month is fine. I know it's not that... He will not let you walk. You're fine. Yeah, I do. But it's back in my home. Just five minutes away from home. You're fine. I used to go to the area a lot so it's cool.

19:46

Can you drop me too? Never mind. This comes back right here. You have a fucking scooter on you man. What's that? I would say it's a scooter. I'm sorry I'm not a student. Yeah, yeah. You're a student. Hey, so tomorrow, right now, my brother's doing lunch, right? I mean, it's the same architecture, right?

21:34

Oh, you can find it outside the square, you know. we need to actually... So right now we are going to build a non -profit business?

23:44

What do you guys use for the backend? The backends? What do you want to use? I feel like messages and phone calls have a different backend from this. You need to...

24:52

emergency alert protocol, emergency alert protocol, or Semiconductor.

28:45

Yes, I've sent you a guitar request. Go ahead. So there's an email.

30:06

Michelle, your IE email is a Michelle email, right? No. I'm using an IE email, so I'm using... I was using an IE email, so I can use another one. How do you know I have another one? Yeah. This is good for a... Jelly? I like it. There's some more. Japanese one. Is that okay? So, let's begin coding guys Where is the, you know, the the docs file?

31:05

Well, as soon as I upload the GitHub repo I mean, I did, so just share it with them like the docs, the research, whatever Pro set up correctly.

35:06

Okay.

36:03

Which email did you use for the Github? The IU email. Gotcha, okay. Do you want me to make a branch? Why? Why are you making a branch?

37:05

because there's nothing here yeah do you have anything okay How's this?

41:01

That's all like East Asian thing That all of the food over there that's all East Asian thing You guys are all welcome.

43:37

I'm afraid to have this cookie. It looks so heavy. So do we need to make like LLM? I mean not LLM, I mean do we need to make a model? What? We need to make a model right? No no no no no, I'm just confusing. Oh no no. Oh no nevermind. What are we working on right now? So you can start coding it as soon as you share the document. I thought I already shared it. Did you? Like, we are not able to access to your, I think, the... Oh, she shared it? Oh, no, that's not it. The cloud thing. No, on the... Did you share it on the GitHub? Oh, we wanted to...

44:46

I just summarized what you get from the cloud in the previous step. And I also put some weak points and strong points here. For the... To shoot her idea from the cloud. This is soaked I'll just look at the back. And we can start pulling. Oh, it will distribute what we want to work on individually.

45:43

And then we can hit push it. Could someone set up the-- Do we need to set up GitHub? No. Setup the... Co -workspace on cloud.

46:18

Push that to get up OpenTelemetry? Yeah, let me set up the Cloud Core. Before that, I guess I need to watch this video too. Catching up. Let me set it up.

47:52

How can I? Oh, it's too sweet. I can feel the sweets. Peace.

48:54

Wow. I'm going to make a lot of rice. I'm going to make a lot of rice. Thank you.

49:56

foreign um Yeah, I don't know.

51:37

I think you can just make folder and you can push Yeah, oh you did?

51:49

Yep, that's it We're not including live video in the platform Capability force for the later.

56:53

Oh Okay Tier 4 Philippines? Is that India Tier 2? Country permission matrix Why is it Nigeria is lower?

57:34

Wait Who's this?

58:54

So do I need to get in? Okay. As -salamu alaykum. I'm sorry, how much? If I like this, what I'm supposed to do? What did I say again? You can go ahead. you can do this. yeah. yeah. yeah. so...

59:46

Could you cross -reference with the deliverables of the cloud case thing that they gave us, and the rubric, and compare that with the WHO and other websites that are, other platforms that essentially are for cloud We can make a JSON file. Like what we're trying to solve. Okay. API, okay. So we can get the tier information first to get a second test. So it's like intake triage. OK. Back in, okay, using Cloud API, okay.

01:00:45

Emergency high and low. Hard execution rules. It's on the cloud. The file that they share with us on the website. The cloud hackathon website. Is there any resources? I think I share the same file with you as well. Yeah. Yeah, same thing. The last three pages or something. Last three pages? You might be looking at page one. There's another page, another tab.

01:01:38

the first step is They cry for the conversation. Okay, the nomadcraft. No, the word sold, I'm not part of that. So the user goes here, asks the question, it specifies, then goes to a gate which decides whether to continue the conversation or not. So it continues to lose the kind of calendar with a state that has distance. Which parts are you?

01:02:29

I'm trying to figure out what one should work on and what I should work on. I can't handle the API for the conversion from the alarm window. The API will just be like... I'm not still sure how it should be one for transcribe, since the transcribe just calls the whisper model, and it transcribes the text, and then it's supposed to pass through the event, so it's the same thing. This one goes directly to the text, and the other one goes directly to the text. I can try to read it on the API. Huh? I can try to read it on the API, too. No, I mean, I can read it on the API. The API is easy for me. I think the other part is the back end. You want to do it from the back end? I think so. That's a hard one.

01:03:29

I think it's fine. I think it's fine. I'll try doing the back end. We still need to figure out, like, I don't think Cloud gave us a good outline. I think we still need to figure out how we're going to build it. Are you a Cloud expert? I mean, you're doing pretty good so far. So... Ishan, will you say something? Oh, he's not. Wait, should I upgrade it?

01:04:30

I tried to add you guys to my project but it says they wouldn't pay money. Yeah. 20 bucks. Yeah.

01:05:41

Oh, okay. I can just share this with the... Okay. Let me see. Oh, wait. How can I...

01:08:31

We didn't post the We're competing against 50 people.

01:11:01

Around 25 teams. So what do you guys want to do? I'm working on the backend. Michelle is working on the APIs and Ishan is working on a research part which will help us do our presentation. Juan, what do you want to work on? Honestly, I don't know. Let me... I think I need a... Let me work the... Let's see. Just give me a second. Let me just figure it out. I think what I can do right now is... Uh...

01:12:03

frontend? wait no, I don't think we need frontend do we need frontend? we need frontend if you want to work on a frontend we'll use react .js if you want to use that if you want to build frontend please do json? react .js json, yeah javascript I never tried it before but let me try it we're not You're wide coding. you're essentially just asking. you need to be certain. be certain. ok. ok. so, michelle, could you get push whatever you have played so far? OK. let me work at that front end. ok. i think I can get that one. but I don't want to cross it yet.

01:13:16

Hey Predator, are you also going to work at the database for the country tiers? Country tier database? So like India, like tier 2? Vishan is working on it. Vishan? Okay. He's working on all the research part. We are only doing the coding. So, Michelle is doing the APIs, I'm doing the backend, you can work on the frontend.

01:13:56

so i know only on the phone only the phone android Okay, okay, let me try Cuz it cuz if you make it Android it took a time like I can do it Yeah, like do we let's do the website first Yeah Because website is what is forcing. So if you have time there, I can work at the Android. Android and Apple use different interfaces. yeah, so Android used like the Yam-- no, not the Yam-- the Darkfire. darkfire and flutter fire. then android studio. yeah, android's-- what do you mean, like, when you're displaying a website? We're not displaying a website.

01:14:56

why would we even develop a website? Wait, why are we doing that? We're not doing a website. no, we're only-- Okay, okay, so let me, okay, let me, actually what I'm envisioning for this thing is, senior. senior. so... can you just move it to here so I can watch it too? You move your chair to the back, please. because I don't want to move it all the way there. i appreciate it. thank you. okay. so... we have our APIs here. our backend. with the mobile... This is connected to... Well, the back end has everything.

01:16:15

That is all. That is what I'm envisioning right now. So... But it's a website or it's in an app? It's not a website, it's not an app. It's a native... It's a service. Yeah. So... For example, you have to call the fire engine or you have to call a police officer. You call 100, right? So we're

using the native mobile and text on our phones instead of a website where they have to go. So you want it to be like call here and they... Yeah, call here and it will begin. That's all. Wait, I thought, like, the thing when I was... What do you mean? Yeah, you don't need to submit anymore. Okay. What do you mean? What do you mean? Like we don't do this? Instead of... Okay, listen. Okay, here's what we want to do.

01:17:11

Like people call, they submit their symptoms, and they respond to that, and they keep... Well... But that's not... This is only the other piece that we have. This is way more user friendly. I feel. OK, you two discuss the definition. Look at this. So for the front end, what I'm thinking was just get the data. And front end is we can do whatever we want. No, so what I was trying to do is just type your country name or whatever. You don't need to do that. No, that's too much work. Wait, if we're doing front end, we need to have a backend in a way. Oh, well, it could call the NBA, but it still needs a backend. Yeah, let's do it. Let me see. But I mean, that would be like for like, we didn't get the-- All right guys, what I want is basically a user who has no knowledge of whatever, and he only has one phone.

01:18:08

And that phone has the ability to call We'll hit that call and they can text Why do we need to? Why don't we just make it a banner? The thing what I'm saying is if you put his data like country data and if you make any animation Why do we want to collect his data? To predict the system. We are making this non -profit. Why do we need to collect this country data? To select the tiers, the database tiers. More tiers. Did you just saw her article 2? What it says was she's using the tiers of the database. In a GitHub. The two.

01:18:56

What it says was, it's usually what she needed, what the thing needs to do, get the country data and get the tier first, and then the one who gets higher tiers, then that causes to become the higher weight on the prediction, the symptoms. So for the future, it becomes... Yeah, so that's what I'm saying. And then like, for that, to get this information, we need, that the person needs to put their country stuff. We don't need for the person to put the country stuff because when you call someone from this person will only have a plus one or So you want, I thought, let me just modify again. So you're trying to make a phone call service or? A phone call service.

01:19:49

So if you do a phone call service, how did you make, like, Wait, then there's no reason for you to use the animated thing that he mentioned a few minutes ago. Or you work at the instruments company. So what I'm thinking is there's two ways you can do it. So the first way is if you call it and then you get the country number, right? That's what you're saying. Automatically get the country number, right? And then why does this person send him the website link Why do they move their symptoms up so high? Too late. Identify his symptoms. Instead of waiting to respond. Let's talk about the symptoms. So then how do you determine like the... Yeah, like why do we do that? It's way more user -friendly than that, than a website.

01:20:41

Like imagine a person in a third world country who basically has no healthcare service and doesn't have a doctor nearby and needs to immediately talk about something that they feel like is critically... constraining their health. So they call this number, 150, which is connected to a chatbot or like we create the agent workflow. It is connected to the backend. And we keep having conversations that we optimize. - We don't have an interface for the doctor to receive. - Yeah, we are doing all of that, yes. - But you want just, you and two, you want dad and dad.

01:21:18

This and the doctor like first we'll gain all the insights Through this through the same thing through a website through the call We can we can have the caller like the person who's calling the user have a call back and Oh, you want the doctor to call them back? Maybe not

the doctor, but like... No, it's something that's realistic for the doctor to call back. No, we're not having the doctor call back. No, we're having the doctor... The doctor has to submit his answer. Yeah, the doctor will submit the answer and the chatbot will call him back and say, hey, this is what the doctor said. Wait, how about like, can you just... Why don't we just send the chatbot to make him a link? Send him a link. The thing with links is the person has to access it and then continue using it. And why do we even need a link?

01:22:16

Like, what would we use the website for? I mean, I'm not talking about the website. The links for the... Wait, oh, if it talks, then we can get the, okay. Okay, ALLM, so if this person says, so maybe prepare some questions, and then if that person answers that, and then it will just automatically get this information So that's what you're saying, instead of using the link system. Yeah, instead of using links and all of that. For example, if the user wants to upload an image, it can tell this on the call, and this service will send a link to on its text. Right, you already mentioned it. You want this person to, you know, so like LLM, I mean not the LLM, like some kind of agent service to send them an image.

01:23:09

email, I mean not an email, any link or text, yeah, text message and then they can post their pictures, right? That is all. And the uploaded picture will go back to the workflow and get analyzed and get logged, which will be sent to the doctor at his own time. And the doctor will input his insights, which will be back into the agent. We'll create a whole different agent for that. And that agent will get the context from this agent and will call the user, saying that this is what the doctor says. Let me ask you another question. So what happens if the doctor diagnoses this person wrong? So what I'm thinking is, do we also make a, you know, like, can we just automatically apply the, like, for example, just maybe country, like, whatever, the...

01:24:05

80 was seven was for like North Korea or whatever like their regulation maybe like oh this this doctor diagnosed this person and then you know that doctor actually give him a like drug stuff and then in this country they're not alone so do they automatically just take it off? We have one database. We're supposed to have a database to fill this. So yeah that's what I'm saying so we need a database for drugs. We're doing the tree ground. Right so. Continue continue. Yeah, so what I'm thinking is, so you said the thing is just make him to send him his picture. All that you need to do is do a call to the number, and the number will either have a conversation with the person based on the symptoms this person is mentioning. And if they want to upload an image, this thing will send them a link. The person will upload the image. Yeah, that is all. And this whole thing will happen. The doctor returning back.

01:25:02

Maybe a week later, it will be an automatic call to this user saying, hey, how are your symptoms feeling? Or do you have any other issues you want to mention? That's just a text message. All text messages. But then I have another question. So whenever, like, if there's, like, tons of people who are trying to do this, then what happens if that... So, for example, like... Let's just say this way. So if this person, like not only one, like maybe to say like 100 or let's say 500 and then give like the doctor, whatever, this guy. There's not just one doctor, there are like hundreds of doctors. Hundreds of doctors, right? Yeah. But how we can allocate those? WHO. world health organization. it's the biggest, most organized organization of doctors, nurses, doctors. okay, okay, okay, okay. so let's go with WHO then.

01:25:59

so did, or we also give them like, wait, so what I'm thinking is, oh, so like maybe hurt and maybe mental or whatever, like the... Yeah, whatever. Like eye or whatever. Do you also make the doctors to be separate? Or no? So like... All doctors have the same knowledge. No, no, like what I'm saying is... Cardio surgeons and heart surgeons. Like, so do you want to be like specified or no? Yeah, it will be specified. That's the tree graph. So that's... Okay, so that's what you guys do. Okay. I'm going to get that working first and then do the trigger.



Wait, where are you going? No, no, no. Do you want one? Do you want one? What were you saying? No, no, no.

01:26:54

Just, you were talking about the tree graph and I thought I went to the top but I forgot that I still need to-- No, tree graph remains constant. No, yeah, but I mean, like, I still have to write the tree graph and I still have to get information for the tree graph and I still need-- Oh, you didn't-- data script. OK, yeah. Cool. How do you-- I have a tool for data scripting if you want to use that. Wait, so you're doing data scraping right now? I mean, we could. We could do data scraping, but we still need to sort the data. That can be done by AI. Wait, so data, what we need is the doctors. So, wait. Sorting can be done by AI with classification. We can't scrape like my exam. I mean I have an extensive scraper tool I got from GitHub and it does really good scraping. Like everything. Like even websites that don't allow scraping. So let me show you.

01:27:54

Do we get the older-- When have you scripted with them? I mean, whatever you give it-- No, but when have you scripted with them? I've got agentic workflow graphs currently so far for building my other thing that I was talking to you about. What websites have you scripted? It's an automatic thing. So you never know what it's going to say? I don't know. I mean, it logs it. I haven't taken a look at it. It logs everything. But still, my opinion should be easier. We can just do a page and then scripts while you're playing. Wait a second. What kind of data we are actually looking for right now? For the tree thingy, I'm thinking like a script while you're playing and then have like an L and just divide everything and make it. No, wait, what I'm saying is, so I think the first one is the country data. You can tier it, right? That thing can be AI. Oh, no, they're just role -based. Yeah. Permission -based.

01:28:54

Permission -based. Permission -based. Because we have to get permission from the government. No, no, no. I don't mean like... I mean like the database, when you authenticate someone, you can handle permissions. So this one is a user. This one has a little bit more privilege. Yeah. So based on that, we would have to filter in the back end who gets access to what. Wait, the... One, two, one. So like that and from there, based on that permission we know like, the main thing that we have is can we give advice, can we not?

01:29:43

And in this case when the doctor records their message, the main thing that we would want to do is hear that provide instructions, you are given, you are given, XYZ. You know. Like, you're doing XYZ. You can't diagnose, something like that. So we can remind them that they can't, they aren't actually allowed to diagnose. Okay. So, aside from that, the other idea that I have with the website is that we could provide links or something. Well, I don't think it's necessary, but it could be useful somewhere. To resources? No, yeah, to the resources for... Local resources. No, that they can obtain a certificate to certify themselves to do it in another country. Okay, yeah.

01:30:43

Resources. They're certified. Right. So they don't necessarily have to be a doctor? Is that what you're trying to... No, no, no, they have to be a doctor, but to provide medical care in our country, they would need these resources. And aside from that... To get onto a database, you mean? Everyone should have access to that, but for this, what it appears here depends on the permission they have and this permission will be based on their location or what they tell us there. So maybe just like this, like, so you agree, then press 1 and then move on to another question. What are you talking about right now? So I'm just talking about like the how this, the phone call works. Did we discard the live version or not? You want to do a live version?

01:31:43

What do you mean the live version? We had the initial idea of a live version and not a live version. Talking to the person? Wait, what are you talking about? So, I thought like, we were just talking, so agent AI, like if you just call the phone number, like for example, like 150. So if you call this, call the 150, then there is a question that, oh, like they just said that this is this, like. No, this is for the doctor. On the AI side, we don't have anything to worry about because what the AI does is just prompt the user Be more specific. And we'll do the knowledge graph. I'm not talking about the AI part. I'm talking about before the AI part. I think this is also supposed to be notified to this person before we move on. What happens if this person likes to say, oh...

01:32:40

In the case of the person, we should just put, depending on whether they belong to a country, we should just specify, reminder we cannot level out in their country. end of the story because why would they need to know they we can just put the disclaimer there but so this they cannot they can offer our system to utilize that's what you mentioned no that person is just trying to get help so in this case let's say they live in a country that doesn't provide that wouldn't that hasn't given information to make medical diagnosis online so this person uh So this person, why does he need to know they, well, he just needs to know the inform. So in this case, we just put a reminder on the top of the page. You, we can't diagnose.

01:33:27

So you mean that, so you're also going to make a, That's what I'm saying. So I just want to make sure that this is not a website, this is a phone call. Yeah, it's a phone call or a text. Hey, for this, can I first test it out with-- can I do some evaluation to test it out? Maybe that tree graph isn't necessary. Oh, what? Maybe that tree graph isn't necessary. No, the tree graph isn't necessary because for the AI to pin down on what questions it needs to ask. For example-- The user enters, My heart is giving me trouble. Oh wait, but that's too wide. Yeah, that's exactly the thing.

01:34:27

That's why we wanted the tree -based search. We wanted the tree -based search. So like, they could ask better questions. Okay. So we map some things. So which part of your body is hers? And if they said like a hand, so like, oh, is it sore? Or is it like, do you guys have any scars on it? That might ask those kind of questions. Asian -Tigay guy asked those kind of questions. And it was follow up. And oh, can you just take a picture of it? And they said... If we would just wait for a few days and doctor will, so doctor would get the... Wait, wait, wait. Right, your colleague wouldn't be able to send the pictures. No, you're... So we should get a technical link? So like, so text... This guy put the picture in here. So the NIA should readjust to come out here. Yeah, for the for the five. That's the one thing. Okay, I have never worked with some of the messages. I've used that time. I am kind of familiar with that. So wait, so send the text is easy.

01:35:25

But before we need a we need a platform though. We need an API. Yeah, right. We don't have enough. I mean, we can test it on a virtual like we can like we can test it right now with email. Email and phone numbers are too little. Yeah, because we need to get the phone number, the nationality number. The thing is, right now that we have a phone number. We can set up OpenCloud. We can set up a phone number. Yeah, on OpenCloud, yes. An agent -open cloud device that just, when you call that number, OpenCloud will start up and start talking back as this whole setup, this whole agent for cloud. And it will do it. Yeah, we can actually host it on OpenCloud. That's not a bad idea. Or NeoCloud. What do you want to host in the... The whole thing. Like the number, the calling service, the message sending. Wait, yeah, oh yeah yeah yeah wait, okay.

01:36:27

So then what we need to do is So we need to give them a voice service. But firstly, we prompt this. The LLM cannot provide you the diagnosis. No, we still need the website for the

doctors. Doctors as well? OK. Yeah, the doctors-- Oh, doctors, we need a website. We need a website for the doctors, yes. Because this calls for the calling from the student. This goes to the database. Yeah, database. Right, right. So this goes to the doctor's website or whatever. Yeah, you know, that's what I do actually, a phone call. I think that I need to work for the, you know, the back end as well as you. So which part do you want to work? Do you want to work at that phone call or at that doctor's website?

01:37:24

I think I should work on the phone code because I'm already working on the api. ok. ok. that would be good. i want to do the integration step or something. yeah, I actually need to figure out that for the function. but how about-- is there any way that we can actually use this number to get the link? Oh, wait. we don't have a number. no. no. but that's what we were saying. oh, yeah. for example, we got-- We just set up the number because I have no idea how to do that. yeah. So what we need to do is use the front number, the plus number to do the configuration for the country and then we also get the tiers, right? The country tiers, right? It's already set up, right? Oh right! We could get it by the country code. Country code and call. Yeah, if we do that then it automatically, we can get the tiers. The tiers goes in the doctor's side, the main one. Yeah, so T or S goes to the doctor's side, but we can also automatically get rid of something like that. Yeah.

01:38:26

Okay, so what you already mentioned is some country, like if the one has like maybe, you know, whatever country it is, but if the place that cannot be diagnosed, the LLM just automatically mentions that, oh, we cannot serve this for your country or whatever. Right? I think that's the... Wait, wait, what example are you talking about? It would be the left one. Which part? The part about-- He's talking about the disclaimer. Yeah, disclaimer. Yeah, but what are you proposing to do instead of just using the funful aside from the fact that that's-- So for example, like-- There's no reason to put a You're having your Newton movement. You're very open to more ideas. Okay. I'm gonna look at the doctor and finish up. I think we got the points. So, like, doctor side, we got a phone call backend.

01:39:27

And then we also got a doctor's website backend. So we got two parts. So for this part, what we need is we need an agentic AI to ask the questions. I should record this. You mean this one? Yeah, for that one. Okay, so you're doing agent AI to make the question. So it's like a chatbot. It's just sound. Yeah, someone didn't get the answers. Okay, okay, okay, okay. Right, right, right. So it's not an agent. No, wait. But this one also needs that, like...

01:40:15

Wait, the part that I'm confusing was, if this one just get the data such as head injury or headache or whatever, please express your illness and say, Oh, I said headache. And then which part of the headache is hers? And I'm like, that means... So here, what I have in mind is, here, it goes here, so it goes to our gate, where it decides, it asks to let you in gates, continue or don't continue? Oh yeah, yeah, yeah, that's the true, yeah, that's the true, yes. And then it goes to continue, so it's like... Okay, yeah, yeah, yeah, that's what you're talking about, yes. And then here, TTS, well, that's what I'm... Okay, and we're timing the conversation. Okay, okay. On the TTS? No, no, not TTS. TTS goes both sides. Sex and speech, it would be... Here. No, technically it would be here. It would be down here. No, here because it would still need to do this. It would only stop on the... Thanks for the speech. Speech to text.

01:41:15

My question is, how do we tear the urgency? What do you mean by tear the urgency? I mean like, I think we also need to... That would be on the doctor's side. There's nothing going on. Because here it's just sending stuff. Here the doctor said it would have to do with hearing. But don't you think that that takes more time? Or whenever they get the data from their phone calls. Here we can send some data, but the maximum we can do is we can't compare. Here we don't have information about the other stuff because this is just sending it

to the database. In the background we're supposed to have access to all of the other calls and everything. So then we can compare and use an algorithm to-- OK. So this one's just data collection. I think we need a better one. Thank you. So data collection.

01:42:18

That's the one. And then this part-- oh, so they can also save the memory at whatever time. Yeah, that's-- Yeah, but also like the-- We're not saving any data, though. Yeah, like-- We're trying to keep it in the loop. We're not going to save data outside of-- I mean, they can also-- No, what I'm thinking here is what I'm only going to-- I'm trying to keep the other one to free. state of just symptoms and I will just send the symptoms. Also that's also the benefits because like if you diagnose this, I mean also you pay for the emergency that it will cost more money for like the phone calls you know always count like the money price of the phone calls always count at the time so you can if you just get the data collection do the data collection and phone call that it will reduce the price of that. Don't you have to get a phone number? What? I'm really curious about that. Oh wait, what happened? Like, how can this person connect back to this guy? That's all in the loop.

01:43:17

How can which person? Like the doctors can connect. The doctor is not connecting. The doctor is not connecting with the user. He's just putting the thing in the website. What he's proposing is that the doctor writes a message, well, speaks a message, and this is a written message. On the website. He's proposing that he gets an additional call, like an automated call, like a voicemail. Oh, wait, wait. Then how about just, why don't we just make a website to this person can just, if this person has a link, Then this guy just directly go to this website. - We are trying to not have any links for the user. - I'm not completely agree with that. - Yeah, me too. - I feel like calls are just better. Everyone in the old age, like people are like-- - I do agree with that point. I did like their website, but I also think, yeah, the doctor does need a website. Maybe they don't have a website. - The doctor needs a website, yes. He's not gonna get a call. - Right, right, no. Oh, here will be more...

01:44:17

No, I just realized that here I have a bit more fail. Yeah, we need a lot of fail there, guys. Yeah. Since they will also send images through the message, text message, and it would be too much away to send and all that stuff. It needs to have a data center. It needs to weigh in. If it weighs too long, it will hang up. Do you also identify the picture brightness or something? No, we cannot identify anything on that side. We won't train anyone else. What happens if that person keeps your own pictures? What? Well, the doctor... We can't handle that on our side. We can't handle that. I mean, if it was a website, we could have, but on a phone call, that's too much work. We just need a simplistic way to connect the doctor to the person who can't reach a doctor. That is all. End of story. Wait, wait. Oh, we will do the same number. Yeah, you're right.

01:45:13

I know you want to do more but at the same time if we do more it will get more I think that phone call is complicated but I don't know yeah the phone call idea like the collection and then the waking website it's a good idea yeah thank you It was my idea, but you know... Okay, okay, okay. You take all the Michelle's ideas. You'll be like, Michelle first on YouTube. Let's make Michelle one of the team, or whatever. No, no. Okay. You are... No, it's your idea. No, it's your idea. So what are we missing? The point you're missing is the... well... I think I can make it. Oh, we still need to figure out how we will do the prioritization queue. We could do it on LLM or I don't know. What do you guys think for the prioritization queue?

01:46:12

Oh, also, we need like expiration. I forgot about this. Expiration. So if a message hasn't been read in a long time. Also, you need to match back country. Match messages to doctors by country. Oh, message after backcountry. ok. yeah. that's-- yeah. all of that is back -end. yeah. so expiration date for messages, they haven't been answered. and we also need to inform the person, therefore, when the message expires and it hasn't been read. we need to

assign specific values for that. what else? We also need priority queues here, I guess. Yeah, probably gives over knowledge for us. We're going to have a warrior, and I'm something to decide which warrior of this is going to get attended. And the doctor's in some research. I think that's the warrior. Who's the person with the crown?

01:47:13

Who? The doctor. No, there's a person with the crown. You remember him? Okay, I think we have a general idea of what is going on. Yeah, I think now we have a better idea of what we're doing. So, Wansuk, what do you want to work on? The website for the doctors is going to be like... Yeah, you can work on the website for the doctors. I don't know if this is necessary for this stage, but we do need some incentives and time for the doctors. What happened if the doctors diagnosed the wrong way?

01:47:59

If a doctor has already attended a patient's aid Patient A should be higher in the priority queue the next time the doctor decides to attempt again. That's too much work. But it should be able to have a follow -up at the doctor's. Wait, wait, wait. I know that, like, doctors... Oh, you mean, like, the patient gets connected to the same doctor. Should be given a higher score to be more likely. Yeah, yeah, yeah. Same country, like, get a higher score. We should have that. We should have that. Higher priority to get into the-- or the second highest priority, based on-- This higher priority is-- Among the three colors, yeah.

01:48:57

We're probably going to score it somehow. Aren't they doctors have like like because for example in South Korea like said like us just after count diagnose South Korean people Unless they got permission like or Can we just say like what was it Guatemala qualified or like us qualified like this this person this doctor shows like oh, oh We are not disclosing any information of the doctor to the user. Oh, right. We don't... No, but we don't need to. No, but shouldn't we display some information like your doctor is this name? Yeah, doctor is this guy. Yeah, that still should be... Oh, yeah, it's just for the professional. Also, the front end, we probably won't implement this, but we do need a fake mechanism to verify right now. Are you a doctor? You need to verify you're a doctor. Right, right, right, right. Are you a doctor?

01:49:57

Yeah, that's the place where I show up. Are you a doctor? Are you a doctor? See that's the thing where... That's the point where I'm saying I'm a doctor I'm pretty sure the doctor would be like Huh? Am I a doctor? Okay, the problem is... There's probably already a law mechanism for that existence. Well, aren't you able to, like, they all got, like, their license number. Yeah, their license number. So please put your license number there. That would be very confidential. No, that is the reason I want to have WHO in here. No, that's why we also need to be, well, at least we need to speak with someone. What is the relationship between? WHO has a database of doctors. They already do have a database. They have a database of doctors. You want to connect to our database of doctors or you want our database to be connected to their? No, no, no.

01:50:54

I want WHO, I want this product to be designed for WHO. Okay. Yeah, so WHO, like, Don't so don't age of very fine. Yeah, cuz we are not the people who are like gonna host the right right right right ask W a chill for like he's is he a doctor and if they you know, they know We could do that, but I'm saying that but they would still need to put their license All of that, but what I'm saying is we are gonna position this product as something for W a chill No, Vaughan has a recording device that's recording everything. I'm literally relying on that. Right now it's recording it. It is recording. It will give me all of the transcripts so that I can practically paste into ChatGPT. I know. So what I'm saying... But now we've got a pretty good picture, like overall picture.

01:51:54

Which is good, okay. William, now I need to change it to a sync, but it's fine. You need to do what? No, a sync. Juan, could you extract the thing that we talked about? I think I just recorded, like, more than hours, I think, too. Yeah, record it. Send it back to me. Yep.